

Project Narrative

CDC-RFA-JG-26-0054: Strengthening global health security through local partnerships in Uganda

FAIRBANKS MEDICAL CENTRE LIMITED

Your health, our mission.

Table of contents

1. Background and approach
2. Evaluation and performance measurement plan
3. Organizational capacity and collaborations
4. Work plan snapshots

1. Background and approach

1.1 Who we are

FairBanks Medical Centre Limited is a Uganda health organisation in Kampala. We run a medical centre and FairBanks Community Reach. Our work starts with families and communities, then moves through community health workers and VHTs, outreach programmes, clinical care, learning with partners, and livelihoods support. Our slogan is simple: Your health, our mission.

The FairBanks Blueprint says strong health systems are built on strong local institutions. It asks us to put people before processes, serve with stewardship, and leave our communities better than we found them. That is why we built the FairBanks Integrated Health Ecosystem (clinical care, community health, training, financing, and partnerships working together) and the FairBanks Community Health Intelligence Platform (FCHIP). FCHIP helps us collect community and facility signals, see patterns earlier, and feed useful information into government pathways.

1.2 Why this CDC partnership matters

CDC wants partners who help keep America safer by stopping health threats closer to where they start. Uganda wants the same thing for its own people: better surveillance links, ready workers, and emergency systems that reach districts and communities. Uganda's NAPHS II and the 2023 Joint External Evaluation point to those same needs.

FairBanks already works in peri-urban Kampala homes, schools, and community spaces. We already see patients at the medical centre. We already collect field data through CHWs, VHTs, and a working FCHIP toolset. What is still missing is a steady last-mile link so that daily community work becomes timely information the Ministry of Health can use. We are not trying to build a second national system. We want to strengthen the one Uganda already leads.

If funded, CDC gains a local partner with real field presence. The Ministry of Health gains better last-mile feed into NISS-aligned pathways and practised detect, notify, and respond loops. Communities get earlier alerts and clearer referrals. FairBanks gets to prove that its tools serve the public system.

1.3 The problem

Uganda sits in a region where endemic, new, and returning infectious diseases keep causing outbreaks. Fast urban growth, crowded peri-urban settlements, and high travel movement raise the chance that a local cluster becomes a national and cross-border threat. People in Uganda and people in America are safer when outbreaks are found early, close to where families live.

In communities where FairBanks already works (Bukoto, Kyebando, Kisaasi, Kamwokya, Kikaaya, and nearby areas), families often reach care late. CHWs and VHTs visit homes and schools, but much of what they see still sits in paper books or separate tools. Clinic records, community reports, lab results, and place or weather signals rarely join one shared picture.

Uganda's 2023 Joint External Evaluation showed real progress on emergency response, labs, surveillance, and workforce. It also named clear gaps: weak information sharing across sectors; limited internet access for surveillance data; incomplete surge plans and staffing at subnational level; uneven use of data to forecast risk; and the need for stronger links from communities and facilities into national emergency and surveillance structures, including work toward a functional National Integrated Surveillance System (NISS).

FairBanks proposes to close that last-mile gap under Ministry of Health leadership. We will help community and clinic signals reach MoH systems faster, train the people who collect those signals, and practise response with district structures so 7-1-7 style timing becomes real on the ground.

1.4 Goal, approach, and outcomes

Goal: Strengthen Uganda's prevention, detection, and response capacities for priority human and zoonotic threats by linking FairBanks Community Reach, FairBanks Medical Centre, and FCHIP tools into Ministry of Health-led surveillance and emergency pathways, so outbreaks are found earlier and contained closer to source.

How we will work: train and equip frontline workers; capture structured community and facility signals; share into NISS-aligned and MoH channels; trigger district and regional response; review timing against 7-1-7; improve; and leave tools ready for government use.

Outcomes we will contribute to:

- Strategy 1: Stronger health emergency systems linked to districts and communities.
- Strategy 2: A more ready public health workforce with better coordination and response skills.
- Strategy 4: Better integrated surveillance that supports earlier detection and response, guided by 7-1-7 style timing.
- Strategy 6: Stronger links between public health systems, clinical care, and communities.

For Strategy 3 (national laboratory systems) and Strategy 5 (border health), FairBanks will support the Ministry of Health and specialised partners. We will help with referral pathways, specimen logistics coordination, private-facility incident reporting where asked, and community risk communication near travel corridors. We will not claim to lead national lab or Points of Entry programmes alone.

Geography: we start in Kampala Capital City Authority catchments linked to FairBanks Community Reach. In Years 1 and 2 we can expand to selected neighbouring districts agreed with the Ministry of Health and CDC.

1.5 Year 1 Component 1 activities by strategy

We build on IDSR and eIDSR, community-based surveillance, CHW and VHT structures, PHEOC incident management, 7-1-7 timing, One Health awareness, and risk communication. Tools will work offline first and respect Uganda data protection and consent rules.

Strategy 1: Health emergency management

- Map FairBanks catchments to district and regional PHEOC structures and agree notification SOPs.
- Run table-top and field drills with CHWs, facility staff, and district teams using 7-1-7 timing.
- Practise incident management for small events, from community alert to district activation.
- Support district-level response coordination for events in FairBanks catchments.
- Document after-action reviews and corrective actions.

Strategy 2: Workforce development

- Train and coach CHWs, VHTs, and selected facility staff on community-based surveillance, event notification, and safe specimen and referral pathways.
- Support One Health briefings with local animal and environmental focal points when districts arrange joint sessions.
- Build a surge roster of FairBanks-linked workers for community investigation and risk communication.
- Align training with Ministry of Health expectations and share materials with MoH and CDC.

Strategy 3: National laboratory systems (support role)

- Strengthen sample referral and transport links from FairBanks Medical Centre and outreach points into MoH-approved laboratory pathways.
- Help MoH track private-facility coverage and routine incident reporting for priority outbreak diseases in our catchments.
- Support biosafety basics at facility sample handling points through SOPs, PPE, and documentation.

Strategy 4: Surveillance

- Deploy and validate FCHIP mobile capture for CHW, VHT, and facility sentinel signals.
- Configure exports and dashboards that contribute to NISS and MoH pathways under MoH rules.
- Train staff on data quality, analysis, and simple visualisation for catchment and district review.
- Use maps and approved weather feeds where they help early warning, with honest limits on what models can claim.

Strategy 5: Border health security (support role)

- Coordinate with MoH and border health partners on risk communication and community surveillance near high-mobility corridors serving Kampala catchments.
- Share community mobility and event signals that support district risk pictures when asked.
- Do not claim to run national Points of Entry programmes.

Strategy 6: Public health programmes and service delivery links

- Strengthen community to facility linkages for priority diseases during routine and outbreak periods, including GHS priorities, HIV, TB, malaria, cholera, Ebola, Marburg, mpox, and immunisation work.

- Support public health campaigns with community mobilisation and data feedback. Routine clinical care is not charged to this award.
- Write SOPs for data use in service and public health decisions, and train teams to use them.
- Provide MoH, CDC, and partners with copies of or access to tools, training materials, and systems developed under the award.

1.6 How this continues national work, and Year 1 timeline

This proposal continues the direction of prior CDC Global Health Security investment in Uganda. FairBanks adds the missing community last mile through Community Reach and FCHIP on the data and feedback loop, configured to feed government systems.

Quarter	Year 1 milestones
Q1	Staffing; MoH and district kick-off; SOP drafts; CHW roster; FCHIP form pack; start EPMP and data management drafts
Q2	Train first cohorts; routine community event reporting; first joint drill; sample referral pathway live
Q3	Expand sites; mid-year data quality review; second drill; MoH data-sharing test exports
Q4	Annual performance package; Year 2 work plan; tool handoff package; surge roster exercise

2. Evaluation and performance measurement plan

About 5 to 10 percent of project funds will support monitoring, reporting, and evaluation. Final indicators will be agreed with CDC after award from the DGHP partner-level list.

2.1 Priority indicator areas

- Surveillance and community mitigation: staff trained in investigation and contact support; risk mitigation actions; risk communication knowledge gains.
- Emergency operations: trainings on rapid response, risk communication, and public health emergency management skills; SOPs for notification and incident management; timely notification toward district activation using 7-1-7 style timing.
- Laboratory support: sample referral completeness and turnaround from FairBanks-linked sites.
- Infection prevention and control: IPC focal person and guideline-based improvements at FairBanks Medical Centre and linked sites.

2.2 Project measures for Year 1

Measure	Baseline plan	Year 1 target
Median days from community signal to district notification	Establish in Q1 to Q2	Move toward 7-1-7 and publish quarterly trends
CHWs and VHTs with complete weekly reports	Roster at kick-off	At least 80 percent of the active roster by Q4
Priority referrals with documented outcome	Baseline audit in Q1	At least 70 percent documented by Q4
Successful MoH or NISS-aligned data package tests	0	At least 2 by Q4
After-action reviews after drills or events	0	At least 2 by Q4

2.3 Methods, reporting, and data protection

- Collect data through mobile forms that work offline, facility registers, training pre and post tests, drill logs, sample referral logs, and dashboard extracts.
- Review weekly for operations, monthly inside the team, and quarterly with CDC. Submit an Annual Performance Report.
- Check quality through form validation, supervisor spot checks, and quarterly data quality audits.
- Use the findings in monthly improvement meetings, share with district and MoH teams, and adjust SOPs and training.
- Plan a Year 2 process evaluation. Consider a later outcome evaluation if funds allow.
- Protect data with role-based access, follow the Uganda Data Protection and Privacy Act, minimise personal data, and de-identify analytics. A detailed data management plan will be ready within six months of award.

3. Organizational capacity and collaborations

FAIRBANKS MEDICAL CENTRE LIMITED (Company No. 80020003843337; TIN 1053370026; NSSF NS043295) is a Uganda company with principal operations in Kampala (Plot 1423 and 1425 Tirupati Road, Fairbanks Medical Centre, Kampala Central Division, Kololo IV, Lugogo). We run three connected pieces of work: FairBanks Medical Centre for outpatient care, diagnostics, pharmacy, and referrals; FairBanks Community Reach for CHWs and VHTs, outreach, maternal and child support, GeriCare, school health, CHIS, and livelihood pathways; and FCHIP, a working platform for mobile capture, sync, dashboards, maps, and secure data links.

This matches the Blueprint idea of one ecosystem. Clinical care, community health, learning, and data belong together. Stewardship means we use every award resource carefully and leave usable tools with government and district partners.

Project Director: Racheal Nabukeera, Managing Director and Co-founder. She brings more than 15 years in Uganda private healthcare leadership and human resources, an MA in Social Sector Planning and Management from Makerere University, and ongoing doctoral study in Management. She also works through Uganda Healthcare Federation links. Contact: info@fairbanksmedicalcentre.org, +256772849258. Website: <https://www.fairbanksmedicalcentre.org/>.

We have not previously managed a large U.S. federal cooperative agreement. We will strengthen segregated ledgers, procurement, timesheets, and audits for this award. We ask CDC to judge us on local presence, technical fit for community surveillance, documented company standing, and our commitment to work under Ministry of Health leadership.

Collaborations if funded

- Ministry of Health (surveillance, community health, emergency operations, digital health).
- District and KCCA health teams covering our catchments.
- Other CDC-funded GHS, HIV and TB, immunisation, and emerging infection partners.
- Community leaders, CHWs, VHTs, schools, and local community organisations in Community Reach.

Key personnel

Role	Person	Notes
Project Director / Authorised Official	Racheal Nabukeera	Overall accountability
GHS Programme Manager	To be hired	Day-to-day delivery
M&E; Lead	To be hired	Indicators, reporting, annual performance
Data / FCHIP Lead	To be hired	Mobile tools and MoH-aligned exports
CHW Supervisor	Existing staff plus surge	Field quality and coaching
Clinical / IPC Focal	Existing Medical Centre role	Sample SOPs and IPC for award-allowed time only

4. Work plan snapshots

4.1 Component 1: Core GHS (Year 1)

Strategy	Activity	Measure examples	By
1	District notification SOPs and 2 drills	Drill reports; time metrics	Q2, Q4
2	Train CHW and VHT cohorts (about 80 people)	Number trained; pre and post scores	Q2 to Q3
3	Sample referral pathway SOPs live	Percent complete referrals	Q2
4	FCHIP surveillance live and MoH export tests	Weekly reports; export tests	Q2 to Q4
5	Corridor risk communication and signal sharing	Sessions held; signals shared	Q3 to Q4
6	Priority-disease community to facility linkage	Completed referral outcomes	Ongoing
M&E;	Detailed EPMP and data plan; quarterly reviews	Reports submitted	Month 6 onward

4.2 Components 2 to 5: Contingency response

Component 2 (\$400,000): surge roster; community investigation and contact support; risk communication; temporary dashboards; district incident management support.

Component 3 (\$500,000): expand surge staffing; multi-district mobilisation under MoH; sample referral logistics; extended risk communication; recovery support.

Component 4 (\$300,000): rapid form updates for new pathogens; sentinel intensification; partner lab referral; special training modules.

Component 5: humanitarian contingency plan. Year 1 federal budget lines total \$3,300,000 across Components 1 to 4. If CDC activates Component 5 under the \$20,000,000 ceiling, FairBanks will support community surveillance and risk communication in crisis-affected groups under MoH tasking.

Closing

The FairBanks Blueprint calls us to build institutions that deserve the trust of the people they serve. Under CDC-RFA-JG-26-0054, FairBanks is ready to serve as a local partner that closes Uganda's last-mile Global Health Security gap under Ministry of Health leadership, so threats are found and contained closer to source. That makes Uganda safer, and it makes America safer too.

Racheal Nabukeera

Managing Director and Co-founder

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